

Urgent Care Clinic - Clinical Note

Date: 10-DEC-2023
Time: 11:40 am

Patient: M. T.

DOB: 11/04/1978 (age 47)

Sex: M

Wt: 82 kg Ht: 178 cm

Attending: Dr. Marcus Vance, MD
Urgent Care Physician
Metro Urgent Care Center
Chicago, IL
NPI: 4491823055
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Chief Complaint: Facial swelling and rash after taking BP medication.

History of Present Illness: 47y/o M with hx of mild hypertension (x 2 yrs), seasonal allergic rhinitis. No prior adverse drug reactions.

Started Cardioril (Cardioril hydrochloride) 20 mg PO once daily on 02-DEC-2023 for essential hypertension. Took 8th dose this morning (~9:00 am).

About 90 minutes later, developed swelling of tongue and ~~lips~~, difficulty swallowing, rash, and shortness ~~of~~ of breath. Came to urgent care for evaluation.

Exam on Arrival:

- Alert but anxious, speaking in short phrases
- Marked perioral and tongue swelling
- Diffuse erythematous maculopapular rash on face, neck, trunk

Vital Signs: - Wheezing on auscultation

BP	HR	RR	SpO ₂	Temp	Peak Flow
96/62 mmHg	106 bpm	22/min	93% (RA)	36.8°C	320 L/min (baseline ~500)

Assessment: 1. Acute angioedema (perioral and tongue) with rash and dyspnea, likely related to Cardioril (cardioril hydrochloride).

Serious reaction - life threatening. Requires observation / hospital transfer.

Management in Clinic:

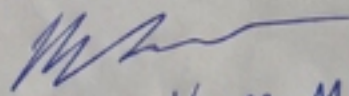
- Methylprednisolone 125 mg IV
- Diphenhydramine 50 mg IV
- Albuterol nebulizer 2.5 mg
- IV Normal Saline 1000 mL bolus

Plan:

- Discontinue Cardioril (do not restart)
- Transfer to hospital Observation Unit
- Monitor for recurrent symptoms
- Patient advised to avoid Cardioril; document as drug allergy.

Outcome: Stabilized after treatment. Swelling and rash began to improve ~~with~~ within ~4 hours. Transferred to Observation Unit for continued monitoring.

Follow-up: Outpatient follow-up with PCP. Consider alternative antihypertensive.


Marcus Vance, MD
10-DEC-2023